Clinical history (de-identified)

Case T062 | Source: open-access case report

A 38-year-old woman, gravida 5, para 0 with 4 prior pregnancy losses, presented to the emergency department at approximately 8 weeks of gestational amenorrhea with a one-week history of mild lower abdominal pain and vaginal spotting. She appeared anxious and uncomfortable. Vital signs showed blood pressure 100/72 mmHg, pulse 110 beats/min, temperature 98.6°F, and respiratory rate 20 breaths/min. Chest examination was unremarkable with equal bilateral air entry. The abdomen was soft and non-tender superficially, but deep palpation elicited mild rebound tenderness in the suprapubic region. Speculum examination showed a mild, thick, brown-colored discharge from the cervical os, and bimanual examination revealed mild tenderness. She had no significant medical comorbidities or known allergies. Her obstetric history included three early pregnancy losses and one mid-trimester loss at 21 weeks. Initial workup showed a serum beta-hCG level of 6,065 mIU/mL. Transvaginal ultrasonography demonstrated a gestational sac measuring 3.8 x 3.6 cm with a crown-rump length of 1.1 cm, located in the right adnexal region, corresponding to approximately 8+5 weeks of gestation. Within two hours of presentation, she developed sudden, severe lower abdominal pain. Given the clinical and radiological findings, a decision was made to proceed with diagnostic laparoscopy for further evaluation and management.